

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

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Finding reasoning paths:

- 1. Behavioral issues and short attention span -> Possible attention-deficit/hyperactivity disorder (ADHD) -> Consider behavioral therapy.
- 2. Hyperactivity and impulsivity -> ADHD -> Behavioral therapy.
- 3. Short temper and aggression -> ADHD or other behavioral disorder -> Behavioral therapy.
- 4. Family history of psychiatric disorders -> Possible genetic predisposition to ADHD or other psychiatric conditions -> Behavioral therapy.

Reasoning Process:

- 1. **Behavioral Issues and Short Attention Span:**
 - The child exhibits behaviors such as not listening, difficulty concentrating, and impulsivity. These symptoms are characteristic of ADHD, which is common in young children and can manifest with hyperactivity, impulsivity, and inattention.
 - ADHD is often treated with a combination of behavioral therapy and, if necessary, medication. Given the child's age, behavioral therapy is often the first line of treatment.
- 2. **Hyperactivity and Impulsivity:**
 - The child is described as more active than other children, frequently interrupts others, and is very forgetful. These are hallmarks of ADHD, particularly the hyperactive-impulsive type.
 - Behavioral therapy is beneficial in managing impulsivity and hyperactivity by teaching the child strategies to control behavior and improve attention.
- 3. **Short Temper and Aggression:**
 - The child has a history of hitting others and being disruptive. These aggressive behaviors, along with the inability to control impulses, suggest ADHD with severe behavioral components.
 - Behavioral therapy can help address these issues by teaching coping mechanisms and improving self-regulation.
- 4. **Family History:**
 - The mother has major depressive disorder, and the father has Graves' disease. While there is no direct indication of a psychiatric disorder in the family history, the presence of a psychiatric condition in a parent can increase the risk of ADHD or other psychiatric issues in the child.
 - Behavioral therapy can help manage the child's symptoms and reduce the risk of escalating behavior issues.

Conclusion:

Given the symptoms of short attention span, hyperactivity, impulsivity, and aggressive behavior, the most appropriate next step in treatment is behavioral therapy. This approach addresses the immediate behavioral issues while also providing a foundation for future interventions if medication is required.

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Behavior therapy
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